

Policy on Clinical Procedures Performed Safely

«Hospital Name»

Document control

Document No.	HCO/COP/POL/07	Version	2.0
Issue No.	01	Review due	One year from implementation
Date created	_____	Date of implementation	_____
Prepared by	Medical Superintendent Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

Clinical procedures are performed based on genuine clinical need, by qualified personnel, with the right patient, procedure, and site verified every time.

1. Purpose

This policy explains how «Hospital Name» decides when a clinical procedure is needed, makes sure only qualified personnel perform or assist it, verifies the correct patient, procedure, and site before starting, and documents what was done.

This policy does not cover informed consent content, patient assessment, or privileging criteria in detail — those are covered in other hospital policies.

2. Scope

This policy applies to all clinical staff who order, plan, perform, or assist in performing clinical procedures at «Hospital Name».

3. Policy standards

«Hospital Name» bases every clinical procedure on genuine clinical need, uses only privileged personnel to perform or assist, follows a documented checklist to verify the correct patient, procedure, and site before starting, takes informed consent where

applicable, monitors patients during and after the procedure, and documents every procedure accurately.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

- Do not perform a procedure unless it is based on the patient's clinical need and consistent with standard treatment guidelines or sound clinical practice.
- Do not perform any procedure — diagnostic, therapeutic, or supportive — without written guidance covering who does it, pre-procedure instructions, the conduct of the procedure, and post-procedure care.
- Do not skip the pre-procedure assessment, which must include at least the patient's vitals.
- Do not let anyone who is not privileged for a procedure perform it or assist in it.
- Do not proceed with any procedure without completing the documented patient/procedure/site verification checklist, using at least two identifiers, one of which is the unique identification number.
- Do not skip the attempt to verify patient/procedure/site in an emergency — where full verification isn't possible, document the exception in the medical record.
- Do not proceed with a procedure requiring informed consent without obtaining it from the person performing the procedure or a doctor from the treating team.
- Do not skip the minimum monitoring parameters — pulse, blood pressure, and respiratory rate — during and after an invasive procedure.
- Do not document a procedure without including the procedure name, who performed it, the salient steps, key findings, and post-procedure care, along with name, date, time, and signature.

5. What we do

5.1 Base every procedure on clinical need

The decision to perform a procedure is based on the patient's clinical needs, following standard treatment guidelines or sound clinical practice.

A qualified medical practitioner decides whether the procedure is indicated.

Where more than one procedure option exists, the choice is based on the likely best outcome, taking the patient's wishes and safety into account.

«Hospital Name» could audit its procedures over time to help achieve the best outcomes.

5.2 Follow written guidance for every type of procedure

Written guidance covers all procedures — diagnostic, therapeutic, and supportive.

The guidance states who performs the procedure, pre-procedure instructions where applicable, how the procedure is carried out, and post-procedure instructions and care where applicable.

Equipment used for procedures is set up and used safely.

Before any procedure, a brief assessment is done that includes at least the patient's vitals.

5.3 Use only privileged personnel

Anyone who orders, plans, performs, or assists in a procedure is privileged to do so.

5.4 Verify the correct patient, procedure, and site every time

Before any procedure, staff use a documented checklist to prevent wrong-patient, wrong-procedure, and wrong-site events.

At least two identifiers are used to confirm the patient, one of which is the unique identification number generated at registration.

Every team member shares responsibility; the person performing the procedure carries the final responsibility. Where a trainee performs the procedure, the supervising clinician carries the final responsibility.

In an emergency, staff still attempt verification; exceptions are documented in the medical record. Patients or their relatives are involved in site marking whenever possible.

5.5 Take informed consent

Where applicable, informed consent is taken by the person performing the procedure or a doctor from the treating team.

If a trainee performs the procedure, this is stated in the consent documentation and the treating doctor supervises.

5.6 Monitor patients during and after the procedure

For invasive procedures, monitoring includes at minimum pulse, blood pressure, and respiratory rate, plus any other parameter the patient's condition calls for.

The extent and duration of monitoring depends on procedure complexity and the patient's other health conditions.

5.7 Document every procedure accurately

Every procedure is documented with the procedure name, who performed it, the key steps taken, the findings, and the post-procedure care given.

Every entry includes name, date, time, and signature.

6. Stop-work authority

Do not perform a clinical procedure that is not based on the patient's clinical need, that lacks written guidance where required, or that is ordered, performed, or assisted by personnel not qualified for that procedure.

Stop-work applies to the elective or non-emergent procedure start. Emergent life-saving procedures continue with the best available qualified help and are documented afterward.

The person who stops tells the treating doctor and the Medical Superintendent the same shift. Refusing an unsafe procedure is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that this policy is followed and appropriately resourced; receives stop-work notifications the same shift.
Treating doctor / clinical team	Decides clinical indication, ensures the correct patient/procedure/site verification checklist is completed, takes or supervises informed consent, and is notified of stop-work the same shift.
Person performing the procedure	Carries final responsibility for the verification checklist and for accurate procedure documentation.
Supervising clinician (where a trainee performs)	Carries final responsibility for the verification checklist and supervises consent documentation when a person in training performs the procedure.
All clinical staff involved	Follow the applicable written guidance; participate in pre-procedure patient/procedure/site verification; share responsibility for correct verification.

8. Quality monitoring

Monitoring area	What is monitored
Clinical indication	Procedures based on clinical need and consistent with standard treatment guidelines or sound clinical practice
Written guidance	Written guidance in place for all types of procedure performed
Patient/procedure/site verification	Documented checklist completed before every procedure, with at least two identifiers including the unique identification number
Emergency verification exceptions	Exceptions documented in the medical

	record where full verification was not possible in an emergency
Invasive procedure monitoring	Minimum monitoring parameters — pulse, blood pressure, and respiratory rate — recorded for invasive procedures
Procedure documentation completeness	Procedure records containing all five documentation elements and all four meta-elements

9. Training and staff acknowledgement

Staff involved in clinical procedures shall be familiar with the written guidance for the procedures they perform or assist, the patient/procedure/site verification process, the minimum monitoring requirements, and the procedure documentation requirements applicable to their work.

I have read the Policy on Clinical Procedures Performed Safely of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to all clinical staff who order, plan, perform, or assist in performing clinical procedures.

11. Abbreviations

Abbreviation	Meaning
SOP	Standard Operating Procedure
WHO	World Health Organisation

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
COP.7.a	Commitment	Section 5.1 addresses basing every procedure on clinical need, practitioner decision on indication, outcome-based choice where options exist, and

		optional auditing of procedures. Stop-work authority (Section 6) also applies to this element.
COP.7.b	Commitment*	Section 5.2 addresses written guidance for all types of procedure, the required content of that guidance, safe equipment set-up and use, and the minimum pre-procedure assessment including vitals. Stop-work authority (Section 6) also applies to this element.
COP.7.c	Commitment	Section 5.3 addresses use of only privileged personnel to order, plan, perform, or assist in any procedure. Stop-work authority (Section 6) also applies to this element.
COP.7.d	CORE*	Section 5.4 addresses the documented patient/procedure/site verification checklist, at least two identifiers including the unique identification number, responsibility allocation, emergency exception documentation, and patient/relative involvement in site marking.
COP.7.e	Commitment	Section 5.5 addresses informed consent by the person performing the procedure or a treating-team doctor, and trainee-performer consent documentation and supervision.
COP.7.f	Commitment	Section 5.6 addresses the three minimum monitoring parameters — pulse, blood pressure, and respiratory rate — for invasive procedures, plus additional

		monitoring as clinically indicated.
COP.7.g	Commitment	Section 5.7 addresses the five documentation elements — procedure name, who performed it, key steps, findings, post-procedure care — and the four meta-elements — name, date, time, signature.

13. Required Records/Evidence Checklist

Clinical indication and written guidance

- Evidence that every procedure performed is based on the patient's clinical need and consistent with standard treatment guidelines or sound clinical practice.
- Written guidance for all types of procedure performed — diagnostic, therapeutic, and supportive — covering who performs the procedure, pre-procedure instructions, procedure conduct, and post-procedure care where applicable.
- Evidence of safe equipment set-up and use for procedures.
- Pre-procedure assessment records showing at least the patient's vitals were assessed before every procedure.

Personnel privileging

- Evidence that anyone who orders, plans, performs, or assists in a procedure is privileged to do so.

Patient/procedure/site verification

- Documented patient/procedure/site verification checklist completed before every procedure.
- Verification records showing use of at least two identifiers, one of which is the unique identification number generated at registration.
- Records showing the person performing the procedure carries final verification responsibility, or the supervising clinician where a trainee performs the procedure.
- Medical record entries documenting exceptions where full verification could not be completed in an emergency.
- Evidence of patient or relative involvement in site marking whenever possible.

Informed consent

- Informed consent documentation signed by the person performing the procedure or a doctor from the treating team.
- Consent documentation stating that a person in training performed the procedure and that the treating doctor supervised, where applicable.

Monitoring

- Monitoring records for invasive procedures showing at minimum pulse, blood pressure, and respiratory rate, with additional parameters recorded as clinically indicated.

Procedure documentation

- Procedure records containing all five documentation elements: procedure name, who performed it, key steps, key findings, and post-procedure care.
- Procedure records containing all four meta-elements: name, date, time, and signature.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. COP.7.
2. WHO Surgical Safety Checklist (reference for patient/procedure/site verification).

Disclaimer

This policy reorganises the supplied COP.7 objective elements and approved plain-language content into policy format. The stop-work text is reproduced exactly as supplied. Mandatory requirements and their stated modal strength have been retained. The exact two-identifier quantity, three minimum monitoring parameters, five procedure documentation elements, and four meta-elements have been preserved throughout.